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—
on the Future of Health in Europe
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Parallelforum IV: Health Determinants –
Nutrition and Food Policy

Too much of the wrong - Nutritional Problems in Europe

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Mad cow disease and Creutzfeld-Jacob disease, the foot-and-mouth disease and nitrofen in turkey and cereals. The food scandals of the past years have made the Europeans very sensitive in terms of food quality. Even so, most do not seem to spend a thought on their daily eating behaviour – in contrast to the experts participating in the Parallelforum VI “Health Determinants – Nutrition and Food Policy”, of the 5th European Health Forum Gastein, Thursday, 26 September 2002.

The figures are alarming: Almost 30 per cent of the untimely deceased under the age of 65 died because of cancer, many because of high blood pressure, overweight or too high cholesterol values. “By means of a healthy nutrition, a lot of deaths could be avoided,” believes Anna Ferro-Luzzi, Director of the WHO Collaboration Centre on Nutrition in Rome. Actually a hundred times more people die because of malnutrition than because of intoxicated food, confirmed Camilla Sandvik of the University of Oslo, who was working for the European Commission on this issue until the end of August.

Not only a problem of developing countries

Europeans lack the consciousness that nutrition is a problem in highly developed countries. “Unhealthy nutrition and the resulting illnesses are a burdens – not only for developing countries,” declared Karen Lock of the London School of Hygiene and Tropical Medicine. The scientist currently does research for the World Health Organization (WHO) to gain data on the global burden due to nutrition based diseases. Contrarily to a former research report, the first “WHO Global Burden of Disease Study” (1990), Lock and her colleagues have split the risk factor “wrong nutrition” into a number of sub-categories. By means of categories, like low fruit and vegetable consumption, obesity or underweight the researchers gained much more detailed and valuable data than their predecessors. In four weeks the study is going to be published. An astonishing detail Lock gave away already in Gastein: “The lacking



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consumption of fruits and vegetables has a greater impact on health than any of us expected.” Derek Yach, WHO-Executive Director for Noncommunicable Diseases and Mental Health, confirmed Lock by giving results of the upcoming WHO report. “To conclude, diseases which are due to malnutrition and lack of exercise, dominate. This should be our starting point.” The WHO is currently revising its work on nutrition and risk factors, in order to develop new guidelines. “We have started a discussion process to develop a WHO-strategy on nutrition, health and against the lack of exercise until 2004,” Yach explained. He appealed to his colleagues and organization to make coherent demands. One, who demands fat reduced nutrition, has to watch out that the food industry does not substitute fat by sugar.

Notes for the EU and national governments

The reasons for nutrition-related diseases are complex. The overall scheme is insufficiently transported by media, Yach criticizes. “Often the issues are torn apart and there are stories on sugar, fat or lack of exercise only.” There are difficulties with national governments as well. Aileen Robertson mentions in a diplomatic way: “Governments change often, that is why they have to be reminded on food policy from time to time.” That food policy can be a success story, Finnish figures prove. The current rate of heart diseases in Russia is comparable to the one in Finland in the Seventies. Finland managed to lower this rate significantly, Robertson reports. There is still a lot left to be done on the health sector. For national governments, the Europe-department of the WHO has developed the CINDI program. CINDI stands for “countrywide integrated noncommunicable disease intervention” and emphasizes food policy as a central measure for controlling and avoiding noncommunicable diseases. “It is our duty to constantly remind the European Commission, that the EU has very clear guidelines”, underscores Robertson.

The whole work of the European Commission in the nutrition sector is based on article 152 (health care) of the Amsterdam contract, which came into effect in May 1999. Unfortunately the measures of the EU are often limited by national legislations, Camilla Sandvik criticizes. “This is why the work of the European Commission exists basically of health demands and the avoidance of diseases.” She presented a catalogue of measures, fully or partly financed by the European Commission, like Eurodiet (2000), the French Initiative (2000) or the White Paper on Food Safety (2000). “The action plan on food policy is not yet published, but the documents exist and build the foundation for work in the nutrition sector”, Sandvik points out. Furthermore web-sites on nutrition have gone online this year and projects on obesity and breastfeeding have started. Sandvik dealt in her speech also with problems linked to other fields of policy, like the recommendation of nutrition experts to eat more fish and the lack of fish on the market. “How can we tell people to eat more fish, if there is not enough fish available?” Sandvik asked. Firstly the fishing sector has to be reformed, only then other measures make sense.

The West-East Divide

Liselotte Schäfer Elinder reported in her speech not only on fishing, but the whole agricultural sector. She explained the reforms of the Common Agriculture Policy (CAP), the most cost-intensive sector of the EU. 90 per cent of the budget for agriculture is spent on subsidies. “This causes a high price level of food in the EU, much higher than on the world market”, explains Schäfer Elinder. Ample proof that price and availability of food play an important role in nutrition, give the Mediterranean countries. Because fruits are cheap and milk products expensive, all segments of the population eat relatively healthy, while in the rest of Europe particularly well educated people eat a lot of fruits and vegetables.

A significant coherence between economical development and public health was also stressed by Jozica Maucec Zakotnik. In her speech, the Slovenian State Secretary for Health focused on Eastern Europe. The West-East divide in Europe was recognizable on a small scale even in regions of her country. Zakotnik developed a package of measures to face this problem, among other things a food safety strategy, nutrition and food action plans for 2003 until 2008 and establishment of a Food and Nutrition Council. "It is very important for politicians and strategists to find out the determinants which have an impact on health and to set them over and over again on the agenda." To the specific concerns of the agricultural sector, the Austrian agricultural economist Helmut Eder of the Division for European Integration Policy and Common Agricultural Policy referred. "It is not true that subsidies interfere with the world market", he claimed.

Time for action

In the Parallel Forum „Health Determinants – Nutrition and Food Policy“ not only scientists and politicians participated, but also representatives of the food industries. Dominique Taeymans, Director of the European Confederation of the Food and Drink Industries (CIAA) believes, that in particular the food safety is a “real challenge”. He demands of the EU food policy “to promote well-balanced nutrition as part of a healthy lifestyle.” Nevertheless should cultural and regional differences be respected and a free and informed decision be left to the consumer. This position was countered by Tim Lobstein, of the British International Organization for Consumer Food Organizations. He believes that a change to a healthier nutrition is bound to the change of the current economical practices, which are backed up by powerful economical interests. “Campaigns can enforce the political pressure so that actions can follow.”